Drug Interaction Bulletin, Fourth Quarter 2026

Reference	PHA-INT-05 v11
Owner	Medicines Information Pharmacist
Approved by	Quality and Patient Safety Committee
Effective date	05/10/2026
Next review	05/10/2029
Applies to	All clinical areas of the Centre Hospitalier Saint-Aubin

1. How to use this bulletin

The pairs below are those most often encountered on the wards. Severity: **Major** - avoid or manage actively, document the decision; **Moderate** - monitor; **Minor** - no action. A pair that is not listed has not been assessed, which does not mean it is safe.

2. Interaction pairs

Medicine A	Medicine B	Severity	Effect	Management
Allopurinol	Azathioprine	Major	bone-marrow suppression	reduce azathioprine to a quarter of the dose
Amlodipine	Simvastatin	Moderate	raised simvastatin exposure	limit simvastatin to 20 mg daily
Apixaban	Aspirin	Moderate	additive bleeding risk	review need for aspirin
Apixaban	Ketoconazole	Major	raised apixaban level: bleeding	avoid
Atorvastatin	Clarithromycin	Moderate	raised atorvastatin exposure	limit atorvastatin to 20 mg daily
Bisoprolol	Verapamil	Major	bradycardia, heart block	avoid
Clopidogrel	Omeprazole	Moderate	reduced clopidogrel activation	prefer pantoprazole
Digoxin	Clarithromycin	Major	raised digoxin level: toxicity	avoid; if essential halve the digoxin dose and check the level on day 3
Digoxin	Furosemide	Moderate	hypokalaemia raises digoxin toxicity	check potassium weekly
Digoxin	Omeprazole	Minor	small rise in digoxin absorption	no action
Digoxin	Spironolactone	Minor	possible small rise in digoxin level	no action
Furosemide	Gentamicin	Moderate	additive ototoxicity and nephrotoxicity	monitor renal function
Levothyroxine	Calcium carbonate	Minor	reduced absorption	separate by 4 hours
Lithium	Furosemide	Moderate	raised lithium level	check lithium level at 1 week
Lithium	Ramipril	Moderate	raised lithium level	check lithium level at 1 week
Metformin	Iodinated contrast	Moderate	lactic acidosis if renal function falls	hold metformin for 48 hours
Methotrexate	Ibuprofen	Moderate	reduced methotrexate clearance	monitor blood count
Methotrexate	Trimethoprim	Major	bone-marrow suppression	avoid
Omeprazole	Clarithromycin	Minor	slightly raised omeprazole level	no action

Medicine A	Medicine B	Severity	Effect	Management
Phenytoin	Fluconazole	Major	raised phenytoin level	check level, reduce dose
Ramipril	Ibuprofen	Moderate	reduced antihypertensive effect, renal impairment	monitor renal function
Ramipril	Spironolactone	Moderate	hyperkalaemia	check potassium and creatinine at 1 week
Sertraline	Tramadol	Moderate	serotonin syndrome, seizures	use lowest doses, monitor
Sildenafil	Isosorbide mononitrate	Major	profound hypotension	contraindicated
Simvastatin	Clarithromycin	Major	raised simvastatin exposure: myopathy / rhabdomyolysis	suspend simvastatin for the whole clarithromycin course
Spironolactone	Trimethoprim	Moderate	hyperkalaemia	check potassium within 1 week
Tramadol	Ondansetron	Minor	reduced analgesic effect	no action
Warfarin	Amiodarone	Major	raised INR, bleeding	reduce warfarin dose by about a third and check INR weekly
Warfarin	Ciprofloxacin	Moderate	raised INR	check INR within 5 days
Warfarin	Clarithromycin	Major	raised INR, bleeding	avoid; if essential check INR within 3 days

3. Governance, audit and training

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

4. Revision history

Version	Date	Author	Summary of change
1.0	05/10/2021	Medicines Information Pharmacist	First issue
2.0	05/10/2023	Medicines Information Pharmacist	Triennial review; format harmonised
3.0	05/06/2025	Medicines Information Pharmacist	Tables updated after clinical audit
current	05/10/2026	Medicines Information Pharmacist	Review and re-issue

Annex A. Implementation notes

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

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Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

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